## 17.2 Croup (Laryngotracheobronchitis)*

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• Mother says, "He has a barking cough and noisy breathing at night."<br>**Objective**<br>• Barking cough<br>• Stridor at rest<br>• Suprasternal recession | **ACTUAL**<br>Ineffective breathing pattern related to laryngeal edema as evidenced by stridor and retractions. | • No stridor at rest within 4 hours. | • Keep him calm on his parent's lap; avoid anything that upsets him.<br>• Give oral or nebulized corticosteroid and nebulized adrenaline as prescribed.<br>• Give oxygen if $SpO_2$ is low.<br>• Avoid examining the throat, as distress can worsen the stridor.<br>• Watch RR, stridor, $SpO_2$ and consciousness. | • Achieved when there is no stridor at rest. |
| **Subjective**<br>• Mother says, "I am scared he will stop breathing."<br>**Objective**<br>• Mother tearful<br>• Child crying | **ACTUAL**<br>Excessive anxiety [parent and child] related to breathing difficulty as evidenced by crying and expressed fear. | • Parent and child look calmer. | • Explain the treatment.<br>• Keep the parent with the child.<br>• Keep noise and stimulation down. | • Outcome reached when both are calmer. |
| **Subjective**<br>*No specific complaint*<br>**Objective**<br>• Upper airway swelling | **RISK**<br>Risk for accidental suffocation as evidenced by risk factors: progressive upper airway edema. | • Any worsening obstruction is caught and treated early. | • Report agitation, drowsiness, cyanosis, or stridor that gets quieter while distress grows.<br>• Keep emergency airway equipment ready.<br>• Never leave him alone. | • Considered met if he does not deteriorate. |

*References for this care plan: [35], [38], [106]. Full entries are listed in the References section of the CPMN book.